

****DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 49

Age/Sex: 70-year-old Male

MRN: 000-TEST-49

Admission Date: 06/15/2026

Discharge Date: 06/18/2026

Attending Physician: Dr. M. Richardson, MD

****PRIMARY DIAGNOSIS****

Acute appendicitis s/p laparoscopic appendectomy

****SECONDARY DIAGNOSES****

- Chronic tophaceous gout (stable)
- Benign prostatic hyperplasia (BPH) with nocturia
- Type 2 diabetes mellitus, controlled

****HOSPITAL COURSE****

Patient 49 presented to Metro Medical Center on 06/15/2026 with RLQ pain, fever to 101.2°F, and elevated WBC (12.8K). CT abdomen/pelvis confirmed acute appendicitis without perforation or abscess. Patient was taken to OR on 06/15 for laparoscopic appendectomy performed by Dr. Richardson. Procedure was uncomplicated; no conversion to open required. Three 5-mm trocars placed at umbilicus, suprapubic region, and midclavicular line; all wounds closed with absorbable sutures. EBL minimal (<50 mL). Post-op course was unremarkable.

Patient tolerated clear liquids on POD#1, advanced to regular diet on POD#2 without nausea or vomiting. Ambulating well. Pain controlled on oral analgesics. Discharged in stable condition on POD#3.

****PHYSICAL EXAM AT DISCHARGE****

Vital Signs: BP 138/82, HR 78, RR 16, SpO2 98% RA, Temp 98.6°F

Abdomen: Soft, mildly tender at trocar sites, no rebound/guarding. Bowel sounds normoactive. Three small bandaged incisions in normal healing phase.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
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Acetaminophen	650 mg	Q6H PRN pain	2 weeks
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Ibuprofen	400 mg	Q8H PRN pain (if no contraindication)	1 week
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Allopurinol	100 mg	q.d.	Ongoing
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Tamsulosin (Flomax)	0.4 mg	q.d. at bedtime	Ongoing
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Metformin	500 mg	BID with meals	Ongoing
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****ACTIVITY & LIFTING RESTRICTIONS****

- No heavy lifting >10 lbs for 4 weeks post-op
- No strenuous activity or abdominal straining × 3 weeks
- Light ambulation encouraged
- Resume normal activities as tolerated after 4-week restriction period

- May shower after 48 hours; pat wounds dry

****DIETARY RECOMMENDATIONS****

Clear liquids → full liquids → regular diet progression has been completed. Patient tolerating regular diet well. No restrictions going forward. Adequate hydration encouraged (6–8 glasses water daily).

****WOUND CARE****

Keep all three trocar sites clean and dry. Remove bandages in 48–72 hours. Wounds may be gently washed with soap and water. Do not submerge in tub/pool × 1 week. Sutures are absorbable and do not require removal. Call if any site shows signs of infection (increasing redness, purulent drainage, warmth).

****FOLLOW-UP APPOINTMENTS****

1. General Surgery clinic with Dr. Richardson — ****in 2 weeks**** (06/30/2026) for post-op wound check and return-to-normal-